Fax

To: QURESHI, IRFAN	**From:** Pam Health at Home - Texas
Fax: 13615741768	**Pages:** 9
Phone:	**Date:** 03-02-2026 5:26 PM
Re: Order(s) for Signature	

Comments:

This fax contains orders for signature.
Please have the physician/clinician noted on this order.
Please sign, date, and return all pages to 1-512-295-7070.
Thank you!

PAM Health at Home – Texas
825 Main Street, Suite 110, Buda, TX 78610
Ph: 512-295-7000; Fax: 512-295-7070

IMPORTANT - PLEASE NOTE: The information transmitted herein is intended only for the person or entity to which it is addressed and may contain confidential, proprietary, protected, and/or privileged material. Any review, retransmission, disclosure, dissemination or other use of this information by persons or entities other than the intended recipient is prohibited. If you received this in error please contact the sender and destroy this facsimile.

Order No:485-26310

HOME HEALTH CERTIFICATION AND PLAN OF CARE

1. Patient's HI Claim No.	2. Start Of Care Date	3. Certification Period	4. Medical Record No.	5. Provider No.
94665048901	02/25/2026	02/25/2026 - 04/25/2026	9555	262009456

6. Patient's Name and Address
CASTILLO, ANGIE (9555)
525 W SERENE DR
Placedo, TX 77977
Phone: (361) 894-2016

7. Provider's Name, Address and Telephone Number
PAM Health at Home
825 Main St Ste 110
Buda, TX 78610-3273
Phone: (512) 295-7000, Fax: (512) 295-7070

8. Date of Birth: 04/24/1952 **9. Sex:** ☐ M ☒ F

10. Medications: Dose/Frequency/Route (N)ew (C)hanged

Medication	Dose	Frequency	Route	Status
Adult Low Dose Aspirin 81 mg tablet,delayed release ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Tablet]	81mg	daily	oral	LS
Amlodipine 10 mg tablet ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Tablet]	10mg	daily	oral	LS
Atorvastatin 20 mg tablet ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Tablet]	20mg	daily	oral	LS
Cholecalciferol (vitamin D3) 50 mcg (2,000 unit) capsule ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Capsule]	50mcg	daily	oral	LS
Cyanocobalamin (vit B-12) 500 mcg tablet ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Tablet]	1	daily	oral	LS
Donepezil 10 mg tablet ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Tablet]	10mg	daily	oral	LS
Ibuprofen 200 mg tablet ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Tablet]	200mg	every 6 hours	oral	LS - PRN/As Needed
Pioglitazone 30 mg tablet ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Tablet]	30mg	daily	oral	LS

11. ICD-9 / 10 CM **Principal Diagnosis**

Code	Code Description	Date	O/E
S42.202D	Unspecified fracture of upper end of left humerus, subsequent encounter for fracture with routine healing		

12. ICD-9 / 10 CM **Surgical Procedure**

Code	Code Description	Date	O/E

Medicare#:

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13. ICD-9 / 10 CM Other Pertinent Diagnosis

Code	Code Description	Date	O/E
Z96.612	Presence of left artificial shoulder joint		
G89.29	Other chronic pain		
E11.9	Type 2 diabetes mellitus without complications		
E78.5	Hyperlipidemia, unspecified		
I10	Essential (primary) hypertension		
F02.B18	Dementia in other diseases classified elsewhere, moderate, with other behavioral disturbance		
G47.33	Obstructive sleep apnea (adult) (pediatric)		
K21.9	Gastro-esophageal reflux disease without esophagitis		
M19.90	Unspecified osteoarthritis, unspecified site		
Z79.01	Long term (current) use of anticoagulants		
Z86.73	Personal history of transient ischemic attack (TIA), and cerebral infarction without residual deficits		
Z79.4	Long term (current) use of insulin		
Z91.81	History of falling		
W19.XXXD	Unspecified fall, subsequent encounter		
Z51.81	Encounter for therapeutic drug level monitoring		

14. DME and Supplies:
DME: Grab Bars, Tub/Shower Bench, Walker

15. Safety Measures:
911 Emergency / Protocol, 911 for Emergency, Anticoagulant Precautions, Assistance with IADLs, DIABETIC PRECAUTIONS, Emergency Plan / Measures, Emergency Plan Developed, Equipment Safety, Fall / Safety Precautions, Fall Precautions, Hazard-Free Environment, Infection Control Standard Measures and Precautions, Keep Pathways Clear, Keep Pathways Clear and Safe, LEFT SHOULDER PRECAUTIONS, Medication Safety / Storage, Safe Disposal of Sharp, Contaminated Materials, Safe in Movements, Safe Transfers, Safety in ADLs, Skin Care, Slow Position Change, Standard Precautions/Infection Control, Support During Transfer and Ambulation, Transfer Precautions, Universal Precautions, Use of Assistive Devices, Walker / Cane

16. Nutritional Req.
Sodium Diet; low, Calorie ADA Diet, No Concentrated Sweets, No Added Salt,

17. Allergies:
No Known Allergies

18.A. Functional Limitations | **18.B. Activities Permitted**

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☐ 1- Amputation ☐ 4- Hearing ☑ 7- Ambulation
☑ 2- Bowel/Bladder (incontinence) ☐ 5- Paralysis ☐ 8- Speech
☐ 3- Contracture ☑ 6- Endurance ☐ 9 - Legally Blind
☐ A- Dyspnea with minimal exertion
☐ B- Other (Specify):

☐ 1- Complete Bedrest ☑ 5- Exercises Prescribed ☐ 9- Cane
☐ 2- Bedrest BRP ☐ 6- Partial Weight Bearing ☐ A- Wheelchair
☑ 3- Up as Tolerated ☐ 7- Independent at Home ☑ B- Walker
☐ 4- Transfer Bed/Chair ☐ 8- Crutches ☐ C- No Restrictions
☐ D- Other (Specify):

19. Mental Status
☑ 1 - Oriented ☐ 2 - Comatose ☑ 3 - Forgetful ☐ 4 - Depressed ☐ 5 - Disoriented ☐ 6 - Lethargic ☐ 7 - Agitated
☐ 8 - Other(Specify):

20. Prognosis: ☐ 1- Poor ☐ 2- Guarded ☐ 3- Fair ☑ 4- Good ☐ 5- Excellent ☐ 6- Other (Specify):

21. Treatments and Orders for Disciplines:

Frequency
SN Frequency: 1W5, 1EOW4 Starting Week of 02/25/2026,DC OT ORDER,PT Frequency: 1W1, 2W2, 1W1 Starting Week of 02/25/2026

Vitals Normal Range
BP Systolic: 90 - 160
BP Diastolic: 56 - 100
Pulse: 56 - 100
Respiratory: 12 - 28
Temperature: 96.5 - 100.5
O2 Saturation: 90 - 100

Vitals Physician Alert Range
Temperature: < 96.5 or > 100.5
Pulse: < 56 or > 100
Respiratory: < 12 or > 28
BP Systolic: < 90 or > 160
BP Diastolic: < 56 or > 100
O2 Saturation: < 90
Fasting Blood Sugar: < 70 or > 250
Random Blood Sugar: < 70 or > 350
Weight: < 163 or > 173
Pain: > 7

SN

BEST PRACTICE FALL PREVENTION:
- Clinician will perform fall assessment and instruct patient /caregiver on fall prevention.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION:
- Clinician to assess/instruct on high risk and other medications to include effects, potential adverse effects, interactions and duplicate therapy, dose route and frequency.

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BEST PRACTICE MONITOR AND MITIGATE PAIN:

- Clinician will instruct patient /caregiver on methods to mitigate pain to include medications and non-pharmacological interventions to decrease pain.

BEST PRACTICE PRESSURE ULCER PREVENTION:

- Clinician will perform validated test Braden to assess patient for risk of development of Pressure ulcers.
- Clinician will perform teaching and training to patient/caregiver for pressure ulcer prevention.

DIABETES:

- SN to assess blood sugar readings Q visit.
- SN to instruct patient and family on signs of hyperglycemia to include: Polyphagia- being hungry frequently, pronounced hunger Polydipsia- beingfrequently thirsty, excessive thirst; Polyuria- frequently urinating; Headaches; Difficulty in concentrating; Blurred vision; Weak and tired feeling;Weight loss; Blood glucose level is more than 180 mg/dl.
- SN to instruct patient and family on signs and symptoms of hypoglycemia to include: Confusion, Dizziness, Feeling shaky, Hunger, Headaches,Irritability, Pounding heart; racing pulse. Pale skin Sweating Trembling Weakness Anxiety

GENERAL: BEST PRACTICE DIABETES:

- Clinician to assess/perform monitoring for the presence of skin lesions on the lower extremities and perform patient/caregiver education on diabetic foot care.

GENITOURINARY:

- Clinician to obtain Urine for U/A and C& S as needed for s/s of UTI via clean catch and notify physician of results.
- Instruct patient/caregiver on signs/symptoms of UTI to report to SN/MD.

SURGICAL WOUND:

- Clinician to assess L shoulder surgical wound, instruct patient/caregiver on s/s of infection to report to MD.

HOSPITALIZATION RISK FACTORS:

- Clinician will instruct patient/caregiver on ways to prevent unintentional weight loss and poor nutrition (high calorie foods, recording (wt/diet), etc
- Clinician will assess and teach patient and/or caregivers regarding declines in mental, emotional, or behavior status and when to report to the MD.
- Clinician to instruct patient and/or caregivers on ways to avoid exhaustion and fatigue to include adequate sleep, nutrition, exercise, etc.

CARDIOVASCULAR - HTN:

- SN to instruct pt/cg on ways to manage HTN such as taking medications as prescribed, increasing physical activity, increasing foods/fluids high in potassium and calcium, managing stress, and avoiding high sodium foods, limit alcohol and monitor blood pressures at home.
- SN to instruct patient/caregiver that uncontrolled HTN can lead to heart attack, stroke, heart failure, kidney complications, vision loss, metabolic syndrome and vascular dementia. SN to instruct pt/cg that HTN is known as the silent killer and symptoms may not be obvious.
- SN to instruct the patient/caregiver on the following symptoms could be signs of a heart attack: chest discomfort, discomfort in one or both arms, back, neck, jaw, stomach, shortness of breath, cold sweat, nausea, or dizziness. Instruct patient/caregiver on signs and symptoms that necessitate calling 911.
- SN to instruct patient/caregiver on Heart Healthy diet.

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PT

PHYSICAL THERAPY:

- Evaluation and Treatment
- Establish Home Exercise Program and Activity Program
- Home safety evaluation and fall prevention/teach fall risk reduction strategies
- Therapeutic Exercises: PROM/AAROM/AROM
- Therapeutic Exercises: Balance, coordination, and/or agility training
- Therapeutic Exercises: Strength training
- Gait Training: Asst device with and without an AD

Assessment Summary

ADMIT of a 73 year old female with a past medical history of-HTN, HLD, OA, CHRONIC PAIN, OSA. Pt underwent total left shoulder replacement on 1/26/26 by Dr Perez at Detar hospital. Pt had decline after going home and was admitted to PAM Rehab 2/15-2/23/26 for IPR therapy. Pt was discharged home with family. Pt had no new medications in the home. Pt lives with niece, who assists with all adl and appt needs. Pt niece assists with medications. Pt is able to check BS independently, reports she monitors BS every other day. Pt skin is intact. Pt waiting for walker to be delivered and is a fall risk due to weakness and unsteady gait. Pt has history of multiple falls. Pt is homebound due to-weakness, unsteady gait, left shoulder restrictions, fall risk, must be accompanied by another person in order to leave the home. Pt to be admitted to PAM HH for SN-education, disease process education and monitoring, medication education, pain management education, pu prevention education. PT to eval and treat-weakness, unsteady gait, left shoulder replacement education and monitoring, fall prevention education. Pt goal for HH is to get stronger, have no falls.

Frequency may be completed with both face to face visits and/or telehealth (if appropriate to meet patient treatment needs).
All qualified disciplines may do non-billable visits to supervise staff and collect oasis data as needed.
Within 5 days of the end of episode, a qualified professional may have a PRN visit to complete a comprehensive/OASIS assessment to establish the need for continuing home health services or discharge.
May accept orders from all physicians caring for the patient.
Any qualified disciplines may perform visit for resumption of care when patient discharged home from inpatient facility.
May resume all care prior to the hospitalization.
PAM Health at Home may discharge patient if remains in facility when certification period expires.
All skilled disciplines may check oxygen saturation prn sob.

I certify that the patient is confined to his/her home and needs intermittent skilled nursing, physical therapy, occupational therapy and/or speech therapy for the next 60 days.
I attest to the following:
1. CASTILLO, ANGIE is under my care.
2. I have received the following patient records from PAM Health at Home:
a. Patient Summary Form
b. Hospital/LTC stay information (if applicable)
c. Home Health Plan of Care
d. And the signature below is my signature
e. These patient records were considered in advance of my certification of the patient's eligibility for home health services under

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Medicare.
I certify that this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy or continues to need occupational therapy for the next 60 days. This patient is under my care, and I have authorized the services on this plan of care and I or another physician will periodically review this plan. I attest that a valid face-to-face encounter has occurred on 02.23.2026 by ENGILBERTO J RAMOS III, MD and it is related to the primary reason the patient requires home health services. PAM HEALTH AT HOME, HOME HEALTH AGENCY, MAY PERFORM SOC ON 02.25.2026.

Risk of Hospitalization

Risks: History of falls (2 or more falls - or any fall with an injury - in the past 12 months); Multiple hospitalizations (2 or more) in the past 6 months; Multiple emergency department visits (2 or more) in the past 6 months; Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months; Currently taking 5 or more medications; Currently reports exhaustion;
Interventions: Referrals - SN,PT; Immunizations - Influenza, Pneumonia; Medication Management - Reconciliation, Assess patients: Knowledge,Ability,Resources & Adherence, Education; Patient/Family Education; Phone Monitoring; Fall Prevention Program; Individualized Patient Emergency Care Plan; Care Coordination (Physicians, Hospitals, Nursing homes...); Tele monitoring
Risk Factors: Discharged from Hospital or Skilled Nursing Facility (M1000); Help with managing Medications needed (M2020); Confusion (M1710); Dyspnea (M1400) ; ADL Assistance Needed (M2102 & M2110)

Psychosocial Assessment

Primary Language -ENGLISH;

Cognitive Status

Cognitive status: Impaired decision-making: failure to perform usual ADLs or IADLs; inability to appropriately stop activities, jeopardizes safety through actions

Advance Directive

Living will; POA; Healthcare representative; Patient/Caregiver reports no Advanced Directives in place

Homebound Status

Patient Homebound Reasons
- Is the Patient Homebound: Yes
- Homebound Reasons: Need assistance for all activities; Residual weakness; Require assistance to ambulate; Unable to safely leave home unassisted; Dependent upon adaptive device(s);
- Patient Chronic and Stable: Yes
- Patient leaving the home require Considerable and Taxing effort : Yes
- Comments:

22. Rehabilitation Potential/Discharge Plans/Goals :

Rehab Potential

Good to achieve stated goals with skilled interventions and patient's compliance with plan of care.

Discharge Plan

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Discharge when medical condition is stable and patient is no longer in need of skilled services

Goals

SN

BEST PRACTICE FALL PREVENTION: - LTG
- LTG-Patient/caregiver will verbalize understanding of interventions to prevent falls by end of episode.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION: - LTG
- LTG-Patient /caregiver will be able to verbalize/demonstrate correct dose, route, and frequency of all medications and will be able to recall adverse effects to report to MD by end of episode.

BEST PRACTICE MONITOR AND MITIGATE PAIN: - LTG
- LTG-Patient/caregiver will demonstrate decreased pain with ambulation as evidenced by increased activity tolerance by end of episode.

BEST PRACTICE PRESSURE ULCER PREVENTION: - LTG
- LTG-Patient/Caregiver will demonstrate/verbalize understanding of methods to prevent pressure ulcers by end of episode.

DIABETES: - LTG
- LTG-Patient FSBS results will remain within physician ordered parameters throughout episode.
- Hypo/Hyperglycemia sign and symptom free by end of episode.

GENERAL: BEST PRACTICE DIABETES: - STG
- STG-Patient/Caregiver will demonstrate ability to perform proper diabetic foot care as evidenced by Keeping a daily log by end of episode

GENITOURINARY: - LTG
- LTG-Patient will tolerate collection of urine without pain and discomfort throughout episode.

SURGICAL WOUND: - STG
- STG-Patient/Caregiver will be able to identify s/s of infection of surgical wound by 3 weeks.

HOSPITALIZATION RISK FACTORS: - LTG
- Patient and/or caregivers will verbalize understanding of ways to mitigate risk of return to the hospital and unplanned ER use.

CARDIOVASCULAR - HTN: - LTG
- Patient/caregiver will be able to verbalize ways to manage HTN by end of episode.

PT

MOBILITY/GAIT: - LTG
- LTG-Patient will demonstrate improved activity tolerance as evidenced by the increased ability to ambulate for 300ft or more with or without an AD and with SBA in order to maximize gait capacity at end of episode.

MOBILITY/GAIT: - STG
- STG-Patient will be able to safely climb up/down the stairs and navigate on multiple surfaces including uneven/and or natural terrain with or without an AD and with SBA in order to be able to safely leave the home to attend MD follow up appointments within 5 weeks
- STG-Patient will demonstrate improved activity tolerance as evidenced by the increased ability to ambulate for 250 ft with or without an AD and with SBA in order to maximize gait capacity within 4 weeks.

ROM: - STG
- STG-Patient will improve PROM of the L shoulder to more than 90° of abduction and flexion to prevent tightness and to preserved joint integrity within 5 weeks.

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STRENGTH: - LTG

- LTG-Patient will demonstrate improved strength of RUE and BLE as evidenced by manual muscle test increase from 3/5 to 4+/5 in order to maximize stability during standing activities at end of episode.

STRENGTH: - STG

- STG-Patient will demonstrate improved strength of RUE and BLE as evidenced by manual muscle test increase from 3/5 to 4/5 in order to maximize stability during standing activities within 4 weeks.
- STG-Patient will demonstrate improved Lower body strength by the increased ability to perform chair sit<>stand for 10x (2 sets) with SBA in order to maximize safety during functional transfer activities within 4 weeks.

23. Clinician's Signature and Date of Verbal SOC Where Applicable: (Electronically Signed)
Verbal SOC Date: 02/25/2026
Signed By: Martinez, Cassandra - RN on 02/25/2026 09:24 AM, Cosigned by Strait, Donna - RN

25. Date HHA Received Signed POT:

24. Physician's Name and Address
Irfan Qureshi MD
2700 CITIZENS PLAZA SUITE 102
Victoria, TX 77901,NPI: 1144510074, License#: P3567,Phone: (361) 574-1775, Fax: (361) 574-1768

26. I certify/ recertify that this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy or continues to need occupational therapy. This patient is under my care, and I have authorized the services on this plan of care and I or another physician will periodically review this plan. I attest that a valid face-to-face encounter occurred (or will occur) within timeframe requirements and it is related to the primary reason the patient requires home health service

27. Attending Physician's Signature and Date Signed:
3/3/26

28. Anyone who misrepresents,falsifies, or conceals essential information required for payment of Federal funds may be subject to fine, imprisonment, or civil penality under applicable Federal laws.